

1. Administrative & Study Identification

Full Study Title: A Phase 1 Study of BMS-986500 as Monotherapy or Combination Therapy in Advanced Solid Tumors
Short Title/Acronym: CA243-0001 / BMS-986500 Advanced Solid Tumors Study
Protocol Number: CA243-0001
NCT Number: NCT06997029
Sponsor Name: Bristol-Myers Squibb
Investigational Product: BMS-986500 (Monotherapy) and BMS-986500 in combination with Palbociclib and Fulvestrant (Faslodex, ATC Code: L02BA03)
Phase of Development: Phase 1
Medical Monitor: Bristol-Myers Squibb Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To assess BMS-986500 as monotherapy in advanced solid tumors and as combination therapy in CDK4/6 inhibitor pre-treated advanced breast cancer. **Secondary Objectives:** To evaluate the safety, tolerability, pharmacokinetics, and initial anti-tumor efficacy of the study drugs across various dose levels and expansion cohorts.

2.2 Background & Rationale To address the critical medical need for individuals with advanced, unresectable solid tumors that have not responded to standard treatments. Specifically, this early-phase study evaluates the safety and effectiveness of the experimental drug BMS-986500, both as a standalone treatment and in combination with standard therapies (Palbociclib and Fulvestrant), to offer new therapeutic pathways for patients with hard-to-treat cancers, including CDK4/6 inhibitor pre-treated advanced breast cancer and CCNE1-amplified ovarian cancer.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional
Control Type: Single-arm/Dose-escalation and Dose-expansion (Experimental Groups)
Blinding: Open-label
Randomization: N/A (Treatment assignments based on specific dose-escalation or expansion cohorts: Part 1A/1B/1C and Part 2A/2B)

3.2 Planned Enrollment & Duration Target \$\$\$: 234 participants
Number of Sites: 21 total locations (including University of Alabama at Birmingham, NEXT Oncology, and START Dallas Fort

Worth)
Estimated Study Start: 2024 / 2025
Estimated Primary Completion: TBD (Study is currently Recruiting / Currently recruiting participants)

4. Subject Selection (Eligibility Criteria)

- 4.1 Inclusion Criteria** 1. Participants must be ≥ 18 years of age. 2. Participants must have histologically confirmed diagnosis of a locally advanced, unresectable, or metastatic solid tumor malignancy. 3. Participants must have a measurable disease per Response Evaluation Criteria in Solid Tumors (RECIST) v1.1. 4. Participants must have a stable Eastern Cooperative Oncology Group (ECOG) Performance Status of 0 or 1. 5. For Part 2A only, participants must have CCNE1-amplified ovarian cancer.
- 4.2 Exclusion Criteria** 1. Participants must not have an active brain metastasis. 2. Participants must not have impaired cardiac function or clinically significant cardiac disease. 3. Participants must not have a bleeding disorder or any history of clinically significant bleeding within the prior 3 months. 4. Participants must not have Grade ≥ 2 peripheral neuropathy. 5. Other protocol-defined Inclusion/Exclusion criteria apply.
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5. Intervention & Treatment Plan

- 5.1 Investigational Regimen Dosage/Frequency:** Administered per assigned cohort protocol. Part 1 encompasses Dose Escalation (1A Monotherapy, 1B Combination, 1C Monotherapy PD Sub-study). Part 2 encompasses Dose Expansion (2A Monotherapy, 2B Combination).
Route of Administration: Oral / IV (As formulated for BMS-986500, Palbociclib, and Fulvestrant). **Duration of Treatment:** Until disease progression, unacceptable toxicity, or withdrawal of consent.
- 5.2 Concomitant & Prohibited Therapy Permitted:** Routine supportive care medications for symptom management. **Prohibited:** Concurrent investigational anticancer agents, specific strong CYP inhibitors/inducers (depending on specific PK protocol guidelines), and concurrent radiation/systemic therapy outside the study scope.
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6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures	:---
:---	:---	Screening	Day -28 to 0 Informed Consent, Medical History, Tumor Imaging, Baseline RECIST v1.1, Safety Labs
Baseline	Day 0	Enrollment, First Dose, Pharmacokinetic (PK) baseline draws	Interim Cycle-based Safety Labs, Efficacy Assessments (RECIST scans), PK/PD sampling, AE

monitoring | | **End of Study** | EOT + 30 Days | Final Vital Signs, Exit Interview, IP Accountability, AE follow-up, Survival follow-up initiation |

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Number of Participants With Adverse Events (AEs), Number of Participants With Serious Adverse Events (SAEs), and Number of Participants With Dose Limiting Toxicities (DLTs) to determine the Maximum Tolerated Dose (MTD) and/or Recommended Phase 2 Dose (RP2D). **Measurement Method:** Clinical laboratory testing, physical examinations, and continuous monitoring graded per CTCAE.

7.2 Secondary & Exploratory Endpoints 1. Objective Response Rate (ORR) and Progression-Free Survival (PFS) measured by RECIST v1.1. 2. Duration of Response (DoR). 3. Pharmacokinetics (PK) parameters: Maximum Observed Serum Concentration ($SC_{\max}$) of BMS-986500, Time of Maximum Observed Serum Concentration ($ST_{\max}$) of BMS-986500, and Area Under the Concentration-time Curve Within a Dosing Interval ($SAUC(TAU)$) of BMS-986500.

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Continuous monitoring evaluated at every cycle visit; collected via clinical assessments and patient self-reporting. **Serious Adverse Events (SAEs):** Must be reported to the Sponsor within 24 hours of investigator awareness. **Data Monitoring Committee (DMC):** Internal Dose Escalation Committee (DEC) will review safety and DLT data prior to advancing dose levels.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Safety Set (all participants receiving ≥ 1 dose of study drug); Efficacy Evaluable Population (participants with baseline and at least one post-baseline scan). **Sample Size Justification:** $N = 234$, determined by the requirements of the standard Phase 1 dose-escalation algorithm and the sizing of the subsequent expansion cohorts. **Handling of Missing Data:** Standard Phase 1 conventions (e.g., missing data generally not imputed for safety; specific rules apply to RECIST and PK evaluations).

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki.

Monitoring Plan: Routine onsite and remote monitoring visits by Sponsor designated clinical research associates (CRAs) to ensure protocol adherence. **Audit Trail:** Secure Electronic Data Capture (EDC) system with full audit trail for eCRF locking and data clarification queries.

11. Study Identifiers & Governance

Primary ID: CA243-0001 **Posting ID:** 695488635764527487 **Registry Number:** NCT06997029 **Sponsor/Lead Organization:** Bristol-Myers Squibb **Study Title:** A Phase 1 Study of BMS-986500 as Monotherapy or Combination Therapy in Advanced Solid Tumors **Official Regulatory Title:** A Phase 1 First-in-human Study of BMS-986500 as Monotherapy in Advanced Solid Tumors and as Combination Therapy in CDK4/6 Inhibitor Pre-treated Advanced Breast Cancer **EMA Regulatory Link (Fulvestrant):** https://www.ema.europa.eu/en/documents/product-information/faslodex-epar-product-information_en.pdf

12. Clinical Framework (Oncology Specific)

Primary Condition: Advanced Solid Tumors (including Advanced Breast Cancer / Malignant neoplasm of breast & Advanced Ovarian Cancer / CCNE1-amplified Ovarian Cancer) **Diagnosis Threshold:** Histologically confirmed locally advanced, unresectable, or metastatic solid tumor malignancy **Medical Methodology:** Investigational Monotherapy and Combination Therapy (w/ Palbociclib + Fulvestrant) **Optimization Target:** Identification of the Maximum Tolerated Dose (MTD) / Recommended Phase 2 Dose (RP2D)

13. Study Procedures & Methodology

Management Pathway: Standard oncology clinical pathway integrated with experimental dosing **Education Protocol (HCP):** Investigator meeting, site initiation visits (SIV), and pharmacy manual training **Education Protocol (Patient):** Informed consent detailing dose escalation risks and strict reporting guidelines **Quality Audit Mechanism:** Periodic independent sponsor QA audits and routine data reviews

14. Observation & Follow-Up Schedule

Total Duration: Throughout the treatment period until disease progression **Onsite Visit Cadence:** Highly frequent during Cycle 1 (e.g., Days 1, 8, 15) for PK/PD/safety, transitioning to day 1 of each subsequent cycle **Remote Monitoring Frequency:** Routine data

reviews performed continuously via EDC **Checklist Review Interval:**
Every cycle prior to dispensing next dose

15. Sign-off & Approval

Role	Name	Signature	Date	:--	:--	:--	:--
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]				
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]		Lead		
Statistician	[Name]	_____	[DD-MMM-YYYY]				